

Galea v. Firkser et al.
[Indexed as: Galea v. Firkser]

Ontario Reports

Ontario Superior Court of Justice,

McDermot J.

March 20, 2013

115 O.R. (3d) 150 | 2013 ONSC 1666

Case Summary

Civil procedure — Discovery — Medical examination — Defence counsel in personal injury action based on alleged soft tissue injuries arranging for defence medical examination by neurologist before plaintiff had delivered any medical reports — Plaintiff's counsel subsequently obtaining reports from orthopaedic surgeon and physiatrist — Defendant moving for order for further defence medical examination by physiatrist — Motion granted — Trial fairness requiring that defendant be given opportunity to respond to reports proffered by plaintiff — Defendant not required to show change in circumstances or to file medical evidence as to need for report. [page151]

The plaintiff brought an action for damages for soft tissue injuries allegedly suffered in a motor vehicle accident. The statement of claim was issued in 2006 and the statement of defence was issued in 2009. Before the plaintiff had delivered any medical reports, defence counsel arranged for a defence medical examination by a neurologist in 2001. The plaintiff subsequently obtained reports from an orthopaedic surgeon and a physiatrist. The defendant moved for an order requiring the plaintiff to attend a further defence medical examination, this time by a physiatrist.

Held, the motion should be granted.

Trial fairness required that the defendant be given an opportunity to respond to the reports proffered by the plaintiff. The fact that the defence might regret using a neurologist instead of a physiatrist for the first defence medical did not mean that this was solely an issue of matching reports. That was especially so where the matter had not been speedily prosecuted by the plaintiff. The defendant did not have to show a change in circumstances or file medical evidence as to the need for the report, as this was effectively the defendant's first opportunity to respond to the plaintiff's medical evidence.

Cases referred to

Bonello v. Taylor, [2010] O.J. No. 4432, 2010 ONSC 5723, 100 C.P.C. (6th) 399 (S.C.J.); *Chiarelli v. Wiens*, 2000 CarswellOnt 280 (C.A.); *Fehr v. Prior*, [2006] O.J. No. 5244, 40 C.P.C. (6th) 381, 154 A.C.W.S. (3d) 326, 2006 CarswellOnt 8443 (S.C.J.); *Francescutto v. Bojsic*, 1999 CarswellOnt 294 (Gen. Div.); *Jones v. Spencer*, 2005 CarswellOnt 1531 (S.C.J.); *MacRae v.*

Dreuniok, [2007] O.J. No. 3283, 52 C.P.C. (6th) 104, 159 A.C.W.S. (3d) 801 (S.C.J.); *Mason v. MacMarmon Foundation*, [2011] O.J. No. 4794, 2011 ONSC 5823, 2011 CarswellOnt 11694 (S.C.J.); *Moore v. Royal Insurance Co. of Canada*, 2006 CarswellOnt 231 (S.C.J.); *Rysyk v. Booth Fisheries Canadian Co.*, [1971] 1 O.R. 123, [1970] O.J. No. 1628, 14 D.L.R. (3d) 539 (C.A.)

Statutes referred to

Courts of Justice Act, R.S.O. 1990, c. C.43, s. 105 [as am.], (2), (4)

Rules and regulations referred to

Rules of Civil Procedure, R.R.O. 1990, Reg. 194, rules 33, 33.02(2), 48.04(1), 53.03(2)

MOTION for an order requiring the plaintiff to attend a further defence medical examination.

George Siota, for plaintiff.

Neil Searles, for defendants.

Endorsement of **MCDERMOT J.**: —

Introduction

[1] This was a motion brought by the defendants for an order requiring the plaintiff, James Galea, to attend at a defence physiatrist medical examination. As the defendants have already obtained a defence medical report in these proceedings, [page152] the requested examination requires leave of the court pursuant to s. 105(4) of the *Courts of Justice Act*¹ and rule 33.02(2) of the Rules of Civil Procedure.² The plaintiff objects; as stated by Mr. Siota during argument, it is the position of the plaintiff that this is nothing other than a case of buyer's remorse and a further examination is both intrusive and unnecessary.

[2] As this matter has been set down for trial, leave to bring this motion is required under rule 48.04(1) of the Rules. When that issue was raised during argument, Mr. Siota on behalf of the plaintiff indicated that he had no objection to leave being granted. The defendants have leave to proceed with this motion on consent.

[3] For the reasons set out below, I have permitted the additional defence medical as requested.

Background Facts

[4] This is an action which can be described as being "long in the tooth". The accident on which the claim is based occurred on September 18, 2004, when a basketball net on the roof of Jeffrey Firkser's vehicle flew off into the path of Mr. Galea's vehicle. The plaintiff's vehicle rolled

over into the ditch; Mr. Galea suffered injuries which he states are severe and have caused ongoing pain and discomfort.

[5] The statement of claim was issued nearly two years after the accident, on September 13, 2006. Thereafter, little took place until the statement of defence was issued on March 6, 2009 and discoveries were held on October 2, 2009. Because of the time it took to get this matter to trial, the matter was administratively dismissed on September 16, 2011; it was restored a month later by consent order of Eberhard J. The matter has been finally scheduled for trial by jury at the sittings in Newmarket commencing May 13, 2013; a pre-trial will be held on April 22, 2013.

[6] This injury was what is colloquially known as a "soft tissue injury" and is largely based upon the plaintiff's complaints of pain emanating from his neck, shoulder, back and chest. Because of these complaints, defendants' counsel at the time, Hans Dickie, Q.C., arranged for a defence medical examination by a neurologist, Dr. Upton, which took place on March 9, 2011. Mr. Siota consented to the examination "on the basis that it will be attended as if pursuant to an Order under Rule 33 of [page153] the Rules of Civil Procedure and Section 105 of the *Courts of Justice Act*".³ Dr. Upton conducted a full physical examination and concluded that Mr. Galea had "recovered from the effects of the accident of September 18, 2004".⁴ The report came down in favour of the defence position and stated that there was no "serious impairment of any important physical, mental or psychological function".⁵

[7] In response, plaintiff's counsel recently obtained two reports, one dated October 25, 2012 from Dr. Rick Zarnett, who is an orthopaedic surgeon, and a second report dated November 5, 2012 from a physiatrist named Dr. Samuel Wong. Dr. Zarnett conducted a physical examination, and concluded that Mr. Galea had "an impairment" as a result of the accident "on an indefinite basis".⁶ Dr. Wong was, if anything, stronger in his opinion; he stated that Mr. Galea had injuries as a result of the motor vehicle accident which "are permanent in nature with the likelihood of improvement being highly unlikely".⁷ It is to be noted that although Dr. Wong's report was prepared in November 2012, it was ultimately not served on defendants' counsel until January 18, 2013.

[8] As well, Mr. Galea has proffered a report from his family physician, Dr. Kakavand dated December 22, 2011; although Dr. Kakavand will not qualify as an expert and cannot provide an opinion, he will testify as to Mr. Galea's present medical condition as set out in that report.

[9] As a result, the defendants now wish a further medical examination, this time by a physiatrist, Dr. Soric. Mr. Searles states that a physiatrist is better able to speak to the issue of muscle and soft tissue pain than is a neurologist and that Dr. Soric's specialty deals with muscle pain which is in issue in this matter. He confirmed to some extent the allegation of "buyer's remorse" insofar as he admitted that he would not have [page154] chosen a neurologist to conduct the defence medical as had his predecessor on the file.

[10] Both counsel did all they could to make their respective positions more palatable. Mr. Siota on behalf of the plaintiff undertook not to call his orthopaedic expert so that there would be no issue of "safety in numbers" of expert witnesses. Mr. Searles, on the other hand, stated that he had an appointment available with his physiatrist on March 27 or April 4, 2012 and that a report would be ready within five business days. He noted that the trial would not be delayed and the report would probably be ready in time for the pre-trial. He also initially undertook to use the Soric report at trial and to not use the Upton report; later, he amended this undertaking on

the record that he would use one report only, but he wished, as had his opponent, the opportunity to choose which report that he wished to use.

Analysis

[11] The authority for a defendant to obtain a medical examination of the plaintiff in a proceeding lies under s. 105 of the *Courts of Justice Act*. Under s. 105(2), where a party's physical or mental condition is in issue, "the court . . . may order the party to undergo a physical or mental examination by one or more health practitioners". A second report may be obtained by motion under s. 105(4). Under rule 33.02(2), a second defence medical examination may be ordered "on such terms respecting costs and other matters as are just".

[12] It is apparent from the reading of the cases that the onus is on the moving party, in this case the defendants, to satisfy the court as to whether a further medical examination is warranted: see *Jones v. Spencer*, 2005 CarswellOnt 1531 (S.C.J.), at para. 14; and *Francescutto v. Bojsic*, 1999 CarswellOnt 294 (Gen. Div.), at para. 16. A second medical report is not lightly ordered; generally, the defence is only entitled to one medical examination of the plaintiff: *Chiarelli v. Wiens*, 2000 CarswellOnt 280 (C.A.).

[13] What are the criteria under which a further medical examination will be ordered? Numerous cases have canvassed this issue. Generally, the issue surrounds trial fairness: see *Bonello v. Taylor*, [2010] O.J. No. 4432, 2010 ONSC 5723 (S.C.J.), at para. 15; *Rysyk v. Booth Fisheries Canadian Co.*, [1971] 1 O.R. 123, [1970] O.J. No. 1628 (C.A.); and *Fehr v. Prior*, [2006] O.J. No. 5244, 2006 CarswellOnt 8443 (S.C.J.), at paras. 7 and 9.

[14] The issue of trial fairness was commented on by R.D. Reilly J. in *Fehr v. Prior*, *supra*, at para. 7, as levelling "the playing field". But it is apparent to me from a review of the cases that it is not [page155] simply a numbers game; trial fairness concerns a defendant having an adequate opportunity of meeting the plaintiff's case in a particular matter. It is trite that a plaintiff may go out and obtain as many reports as he or she wishes; consent is unnecessary as they are arranging for their own reports and their own examination. A particularly extreme example is *Francescutto v. Bojsic*, *supra*, where the plaintiff produced some 16 medical reports, but the defendant was limited to one defence medical report. Accordingly, assessing trial fairness involves an inquiry as to whether an additional medical report is necessary to permit the defendant to sufficiently answer the plaintiff's medical reports submitted by him or her for use at trial; D.M. Brown J. stated, at para. 15 of *Bonello*, that the purpose of a medical examination under s. 105 is invariably "done in order to make available for trial an expert report that responds to a report filed by the plaintiff". Later, relying upon *Rysyk*, *supra*, D.M. Brown J. states:

The right to respond to a plaintiff's expert report is a substantive one, involving principles of trial fairness, as the Court of Appeal made clear in *Rysyk v. Booth Fisheries Canadian Co. Ltd.*:

It is plain that, if the defence proceeds to trial without the advantage of the examinations sought, it could be seriously prejudiced. Fairness in the conduct of this litigation seems to demand the defendants have the right now contended for, as, otherwise, the opinion of the plaintiff's expert in psychiatry and the plaintiff's own evidence would not be subjected to what is probably the best test and to a very great extent go unchallenged.

[15] This is confirmed by most, if not all of the cases cited by counsel. None of the cases cited, as far as I could see, involved a situation, as in the present case, where a defence medical was obtained *prior* to those submitted by the plaintiff. In the present case, unlike any of the other cases that were cited to me, the first report produced was that of the defendant, who retained Dr. Upton to conduct the physical examination of the plaintiff. Up to the date of that report, as far as can be determined from the evidence supplied to me, there had been no medical evidence whatsoever produced by the plaintiff. Even the report from the plaintiff's family doctor, Dr. Kakavand, postdates the report of Dr. Upton. The medical report obtained by the defendant does not respond to anything other than the plaintiff's complaints of chronic pain resultant from the accident. As such, if trial fairness involves a defendant responding to a medical report proffered by the plaintiff, these defendants have not had an opportunity to do so.

[16] I expect that the response of plaintiff's counsel may be that this is the defendants' problem; they did not have to [page156] obtain the Upton report, and once they elected to do so, they were limited by s. 105 and Rule 33 to that report. As this was not placed in evidence before me, I can only speculate as to the reason why Mr. Searles' predecessor elected to obtain the report prior to the plaintiff doing so, but one motivating factor may be the inordinate amount of time that this plaintiff took to move the matter towards trial. It is the plaintiff's responsibility to bring a matter to a conclusion, and where six years had elapsed between the accident and the date the report was requested, the defendant may be forgiven for acting pre-emptively by obtaining the Upton report. The issue is whether, in light of the two medical reports now served on the defendant, the latest of which (and the one to be used at trial) only having been served in mid-January of this year, the defendant is to be prevented from responding to these reports by his actions in obtaining the earlier report from Dr. Upton in September 2011. I will examine this in light of a number of issues raised by plaintiff's counsel in argument.⁸

Delay of trial

[17] Initially, the concern expressed by plaintiff's counsel in his materials was that the proposed expert's report would delay the trial and thereby prejudice the plaintiff. Notwithstanding the fact that Mr. Searles announced that early appointments were available for Dr. Soric, this ground was not abandoned by Mr. Siota. He noted that a responding report may be necessary to Dr. Soric's report. This again may result in a delay of trial.

[18] That indeed may be the result; however, I have to return to the original concern expressed above. The purpose of a defence medical is to respond to the plaintiff's medical reports to be used at trial. The present report which has been prepared cannot respond to Dr. Wong's report because it was prepared well before Dr. Wong had even examined Mr. Galea. In fact, the orthopaedic surgeon retained by the plaintiff did review the report of Dr. Upton in the preparation of his report. I do not find that a further report will necessarily delay the trial or prejudice the plaintiff in this matter. If it does delay matters, it is apparent to me that the prejudice to the plaintiff of delaying trial is outweighed by the prejudice to the defendants resulting from [page157] their inability to respond to the plaintiff's medical evidence. Moreover, in view of the plaintiff's delays in prosecuting this matter, I do not find it to lie in the mouth of the plaintiff to complain of delay at this time, should the plaintiff elect to produce a further responding report in this matter.

Change in circumstances

[19] Plaintiff's counsel relied upon a number of cases which stated that some sort of change in circumstances was necessary in order [to] warrant a further report. See, for example, *Mason v. MacMarmon Foundation*, [2011] O.J. No. 4794, 2011 CarswellOnt 11694 (S.C.J.), at para. 46. Mr. Siota states that there is no change in circumstances since the first report was served which would warrant a new report.

[20] I agree with plaintiff's counsel that there is no change in circumstances demonstrated on the evidence before me. However, as noted by D.W. Brown J. in *Bonello*, a change in circumstances may not be necessary where a party is responding to a new report which has recently been served in the litigation; see para. 16(i) of that case. In *Mason v. MacMarmon Foundation*, *supra*, for example, Boswell J. permitted a neuropsychological examination in order to reply to a new report proffered by the plaintiff without there being a demonstrated change in circumstances: see para. 49 of that decision.

Matching report ("buyer's remorse")

[21] As mentioned by Boswell J. in *Mason*, the failure to obtain a report in a particular specialty is not, on its own, grounds for an additional defence medical report. Where the failure to obtain a specialist's report is [at para. 47] "a function of the defendant's tactical choices", the matching of a report will not necessarily be grounds for a second defence medical. And I agree with Mr. Siota that there appears to be little distinguishable difference between a neurological assessment and a physiatrist's assessment of the pain issues; both carry out a physical examination; both opine on pain issues and the only difference appears to be the issue of whether the neurologist looks at the muscular pain complained of by the plaintiff.

[22] However, the prohibition against mere "matching" of reports only applies, as pointed out by Boswell J., where it [at para. 47] "will not render the trial less than fundamentally fair". That was the case in *Mason*, where there were numerous reports before the court. And that is unlike the present case; in this case, the jury will, based upon the undertakings by counsel, be exposed to only two experts providing their respective [page158] opinions. There is, in the present case, none of the "hallmarks of excess" as so aptly stated by DiTomaso J. in *MacRae v. Dreuniok*, [2007] O.J. No. 3283, 52 C.P.C. (6th) 104 (S.C.J.).

[23] In light of this, to fail to match the Wong report will render the trial unfair. That is because, as noted above, the defendants will not have the opportunity to respond to Dr. Wong's report because it was not in their expert's hands when Dr. Upton completed his report. The defendants are entitled to make their defence to the plaintiff's claim, and their evidence is necessarily responsive in nature. As such, a responding physiatrist's report would appear to be warranted. Although there may have been a tactical error made by defendants' counsel in obtaining a neurological report, that does not override the fact that trial fairness requires that the defendants have an opportunity to respond to the reports proffered by the plaintiff very late in the day.

Evidence of medical need for a further report

[24] A number of cases note that without evidence of medical need for a further report, the defendant has not made out sufficient necessity to permit a further medical report to be filed: see, for example, *Moore v. Royal Insurance Co. of Canada*, 2006 CarswellOnt 231 (S.C.J.) and

Fehr v. Prior, supra. For example, in *Moore*, at para. 7, Valin J. states that, "[t]o obtain a second physical examination of a plaintiff, the defendant must clearly demonstrate the need for a second examination and put before the court in some form the reasons and opinion of the first examining doctor as to the need for such examination". It appears from this case that where the issue is within the expertise of the first examining physician, a further report by another type of specialist will not be ordered.

[25] It is acknowledged that this type of evidence is lacking in the present case. However, Dr. Upton would have hardly turned his mind to a second report when his examination was the first and, at the time, only medical report being produced in the litigation. Moreover, this is not a situation, as in *Moore*, where the expert's report is intended to corroborate the first report; Mr. Searles has agreed that only one report will be used at trial. Finally, it may be that the reports from the plaintiff's experts themselves call for a second examination in this case; those reports indicate that the plaintiff's complaints have credibility and are of long-lasting effect and, as such, defendants' counsel should have the right to respond to those conclusions of fact. [page159]

Conclusion

[26] In my view, this is essentially an issue of trial fairness. To prevent the defendants the opportunity to obtain a further defence medical examination would work an injustice, as the defendants would otherwise have no opportunity to adequately respond to the plaintiff's medical evidence as provided for under rule 53.03(2). The fact that there may be some regret at using the neurologist rather than a physiatrist does not mean that this is solely an issue of matching reports. This is especially so where the matter has not been speedily prosecuted by the plaintiff and where there are only intended to be one report by each party relied upon at trial. Even if the trial is delayed, I find that the defendants will be prejudiced by the inability to respond to the plaintiff's expert report, only served on the defendants in January of this year after a hiatus of over eight years from the date of the accident. And no change in circumstances or medical evidence as to need for the report need be filed where this is effectively the defendants' first opportunity to respond to the plaintiff's medical evidence filed in this proceeding.

[27] Accordingly, the defendants will be entitled to a further defence medical examination and report as requested in the notice of motion.

Disposition

[28] As such, there shall be an order to go as per (a), (b) and (c) of the defendants' notice of motion. The order for leave to proceed with this motion is noted to be on consent.

[29] The parties may speak to costs, with the defendant providing costs submissions and then the plaintiff on a ten-day turnaround. Costs submissions to be no more than three pages in length, excluding any offers to settle or costs memoranda.

Motion granted.

Schedule "A"

- (i) The party seeking the order for a further examination must demonstrate that the assessment is warranted and legitimate, and not made with a view to delaying trial, causing prejudice to the other party, or simply corroborating an existing medical opinion;
- (ii) A request may be legitimate where there is evidence that (i) the party's condition has changed or deteriorated since the date of a previous examination, (ii) a more current assessment of the plaintiff's condition is required for trial, (iii) the plaintiff served specialist reports from new assessors after the defendants had conducted their medical [page160] assessments, or (iv) some of the party's injuries fall outside the expertise of the first examining health practitioner;
- (iii) Some cases take the view that the need for a "matching report" -- i.e. a report from a defence expert witness in the same specialty as a plaintiff's expert -- is not, in and of itself, a sufficient reason to order a further defence medical. In the circumstances of the present case I need not wade deeply into that question. That said, I would venture that trial fairness should operate as the guiding principle in this area, so if the plaintiff has decided that expert evidence from one specialty based on an examination of the plaintiff is relevant to the adjudication of her claim at trial, courts should be loathe to deny the defence a fair opportunity to respond with expert evidence from the same specialty based on an assessment of the plaintiff. Ordering further examinations may be just where they are necessary to enable the defendant fairly to investigate and call reasonable responding evidence at trial;
- (iv) Where the request is for the examination of the plaintiff by a person who is not a health practitioner, such as a rehabilitation expert, the defendant must demonstrate that the proposed examination is necessary as a diagnostic aid to the health practitioner who is conducting the defence medical examination;
- (v) A request for a second examination must be supported by sufficient evidence to persuade a court of the need for the further examination. What constitutes sufficient evidence will vary from case to case. Some cases have suggested that need must be established by filing medical evidence, such as an affidavit from the first examining physician recommending a further examination by a health practitioner competent in another specialty. In other instances an affidavit from a lawyer or law clerk attaching medical reports has been utilized by the court. But, at the end of the day, determining whether the nature of the evidence filed is sufficient remains essentially an exercise of judicial discretion;
- (vi) While fairness, or "creating a level playing field", may constitute a legitimate reason for ordering a second examination, someone with knowledge of the evidence in the case must provide evidence of unfairness for the court to consider; and,

(vii) A court should consider whether the request for a further examination would impose an undue burden on the plaintiff in light of the number of examinations already conducted of her by the defence.

Notes

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- 1 R.S.O. 1990, c. C.43.
 - 2 R.R.O. 1990, Reg. 194.
 - 3 See the correspondence from Mr. Siota to Mr. Dickie dated January 21, 2011 attached as Exhibit "C" to the affidavit of Paul Veuglers sworn March 12, 2013.
 - 4 Report of Dr. Upton dated March 21, 2011 and attached as Exhibit "G" to the affidavit of Andrea Geddes sworn March 4, 2013, at p. 9.
 - 5 Report of Dr. Upton, at p. 10.
 - 6 Report of Dr. Zarnett dated October 25, 2012 and attached as Exhibit "H" to the affidavit of Andrea Geddes sworn March 4, 2013, at p. 8.
 - 7 Report of Dr. Wong dated November 5, 2012 and attached as Exhibit "J" to the affidavit of Andrea Geddes sworn March 4, 2013, at p. 8.
 - 8 It is to be noted that many of the criteria that I will be discussing were well summarized by D.M. Brown J. in *Bonello*, *supra*, at para. 16 of the report. Those criteria are a useful guide and for reference are set out in Schedule "A" to this endorsement.